Dental Problems

UP-07 | Universal Patient Care

Quick flow

01 Assess Airway + Cause [EMT / AEMT / Paramedic]

Dental pain • jaw pain • trauma • bleeding • swelling • secretions • voice change

02 Airway-Risk Infection? [EMT / AEMT / Paramedic]

Floor-of-mouth or submandibular swelling • tongue elevation • drooling • stridor • trismus • rapid progression

03 Adult Jaw Pain / Anginal Equivalent? [EMT / AEMT / Paramedic]

Acquire 12-lead ECG • cardiac monitor • transition to adult cardiac protocol

04 Dental Hemorrhage? [EMT / AEMT / Paramedic]

Direct pressure • persistent socket bleeding: AEMT/Paramedic topical TXA

05 Permanent Tooth Avulsed? [EMT / AEMT / Paramedic]

Replant promptly when eligible • otherwise preserve in proper medium • never replant a primary tooth

06 Analgesia + Reassess [EMT / AEMT / Paramedic]

Adult UP-11 or pediatric PM-04 pain control • cardiac monitoring with medication • repeat airway, bleeding, pain, and vital signs

Overview

- Most dental complaints require focused assessment, bleeding control, analgesia, and dental follow-up. Airway-threatening swelling, uncontrolled hemorrhage, significant facial trauma, and anginal-equivalent jaw pain require immediate transition to the appropriate emergency pathway.
- An eligible avulsed permanent tooth should be replanted promptly when it can be done safely. Never replant a primary tooth.

Provider-Level Actions

Higher provider levels include the actions listed for the preceding levels.

EMT

- Assess airway patency, ability to handle secretions, voice, oral and facial swelling, trismus, bleeding, trauma, pain, fever, and complete vital signs. Suction blood or secretions as needed and position the patient to protect the airway.
- For floor-of-mouth or submandibular swelling, tongue elevation, drooling, inability to handle secretions, stridor, trismus, toxic appearance, or rapidly progressive swelling, request Paramedic support, prepare airway equipment, minimize oral manipulation, and begin prompt transport.
- For adult jaw pain or another anginal-equivalent complaint, acquire and transmit a 12-lead ECG, apply cardiac monitoring, and transition to the applicable adult cardiac protocol. Do not obtain an ECG routinely for isolated tooth pain with a clear dental source.
- Control dental bleeding with suction as needed and firm direct pressure using folded gauze placed over the socket. Have the alert patient close the teeth over the gauze while maintaining airway safety. EMT personnel do not administer topical TXA under this protocol.
- For an avulsed permanent tooth, handle only the crown. If contaminated, gently rinse with milk or normal saline without rubbing or scrubbing the root. When the patient is conscious, cooperative, protects the airway, the tooth is intact, and significant jaw or socket fracture is not suspected, gently return the tooth to its socket without force and have the patient hold it in position by biting on gauze.

- Never replant a primary tooth. If replantation is unsuccessful or contraindicated, place the tooth in commercial tooth-preservation solution, milk, normal saline, or the patient's saliva in a closed container. Do not store the tooth in the patient's mouth, on ice, or dry.
- Provide nonpharmacologic care. For patients age 16 years or older, use UP-11 Adult Pain Control; for patients age 2 through younger than 16 years, use PM-04 Pediatric Pain Control; for patients younger than 2 years, contact Medical Control for analgesic direction. Apply cardiac monitoring when medication is administered and reassess pain, airway, bleeding, vital signs, and treatment response.

AEMT

- For persistent tooth-socket bleeding despite firm direct pressure, saturate gauze or cotton with TXA 1 g/10 mL, pack it into the socket, and maintain direct pressure with the patient closing the teeth for 20 minutes. Continue suction and airway protection as needed. Do not administer IV/IO TXA solely for dental bleeding.
- Establish IV access only when required for parenteral analgesia, hemodynamic instability, airway management, or another specific clinical indication. Do not routinely establish IV access or administer fluid for an isolated dental complaint.
- Administer analgesia under the age-appropriate pain pathway, maintain cardiac monitoring after medication administration, and reassess pain, bleeding, airway, vital signs, and medication response.

Paramedic

- Interpret the 12-lead ECG and cardiac rhythm for adult jaw pain or another anginal-equivalent complaint, and treat identified ischemia or dysrhythmia under the applicable adult cardiac protocol.
- For progressive oral, submandibular, or floor-of-mouth swelling with airway compromise, maintain spontaneous ventilation whenever possible, prepare suction and a difficult-airway plan, and transition to the adult or pediatric airway protocol without delaying transport.
- Administer topical TXA and parenteral analgesia as indicated, using UP-11 for patients age 16 years or older and PM-04 for patients age 2 through younger than 16 years, with the medication-specific airway, respiratory, hemodynamic, and cardiac monitoring required by the applicable pathway.

Indications

- Dental or jaw pain, oral or tooth-socket bleeding, dental infection or swelling, fractured tooth, or avulsed tooth.

Contraindications

- Do not replant a primary tooth.
- Do not attempt field replantation when the patient cannot protect the airway, is uncooperative, has significant associated facial or jaw trauma, has a suspected socket fracture, or the tooth cannot be inserted gently without force.
- Do not administer topical TXA to a patient with a known hypersensitivity to tranexamic acid.

Assessment | Focused dental and airway assessment

- Onset and source of pain; tooth versus jaw location; trauma; bleeding; fever; swelling progression; difficulty swallowing; drooling; voice change; trismus; dyspnea; chest, arm, back, or epigastric discomfort; diaphoresis; nausea; and syncope.
- Inspect the face, jaw, mouth, floor of mouth, tongue position, visible tooth or socket, bleeding source, and ability to handle secretions without aggressive probing or manipulation.
- For tooth avulsion, determine whether the tooth is permanent, time out of the socket, dry time, storage medium, root integrity, contamination, and associated facial or neurologic injury.

Treatment steps

- Stabilize the airway, suction blood or secretions, control hemorrhage, and identify rapidly progressive infection or associated facial trauma.

- For adult jaw pain or another anginal-equivalent complaint, acquire/transmit a 12-lead ECG, apply cardiac monitoring, and use the appropriate cardiac protocol.
- Control socket bleeding with folded gauze and direct pressure. If bleeding persists, an AEMT or Paramedic may apply topical TXA 1 g/10 mL on gauze and maintain pressure for 20 minutes.
- Replant an eligible avulsed permanent tooth promptly and gently. Never replant a primary tooth. Preserve a tooth that cannot be replanted in commercial solution, milk, normal saline, or saliva in a closed container.
- For patients age 16 years or older, provide analgesia according to UP-11; for patients age 2 through younger than 16 years, use PM-04; for patients younger than 2 years, contact Medical Control. Apply cardiac monitoring when medication is administered and reassess airway, bleeding, pain, vital signs, and treatment response.

Medications

Tranexamic Acid - topical dental hemorrhage

AEMT/Paramedic: TXA 1 g/10 mL applied topically to saturate gauze or cotton; pack the bleeding socket and maintain direct pressure for 20 minutes.

- Use only after bleeding persists despite firm direct pressure.
- EMT administration is not authorized under UP-07.
- Do not administer IV/IO TXA solely for dental bleeding.

Analgesia

Age 16 years or older: use UP-11 Adult Pain Control. Age 2 through younger than 16 years: use PM-04 Pediatric Pain Control. Younger than 2 years: contact Medical Control.

- Apply cardiac monitoring when analgesic medication is administered.
- Reassess pain, airway, bleeding, vital signs, and medication response.

Warnings / precautions

- Oral or facial swelling may progress rapidly. Drooling, inability to handle secretions, tongue elevation, stridor, trismus, voice change, or rapidly progressive submandibular swelling requires prompt airway preparation and transport.
- Do not probe or attempt to drain a dental abscess in the field.
- Do not confuse anginal-equivalent jaw pain with an isolated dental complaint. Obtain an ECG for adult jaw pain or another anginal-equivalent complaint.
- Do not touch, rub, or scrub an avulsed tooth root. Never replant a primary tooth or force a permanent tooth into the socket.
- An avulsed or loose tooth and blood in the mouth may obstruct the airway, particularly when mental status is impaired.

Clinical pearls

- Immediate replantation offers the best chance of retaining an avulsed permanent tooth when the procedure is safe and the tooth can be inserted gently.
- If replantation cannot be performed, preventing the root from drying is time critical. Commercial preservation solution or milk is preferred when immediately available.
- Document the tooth type when known, time of avulsion, estimated dry time, storage medium, replantation attempt and result, bleeding treatment, associated injuries, and destination notification.

Special populations | Pediatrics - younger than 16 years

- Carefully distinguish a permanent tooth from a primary tooth. Never replant a primary tooth because of potential injury to the developing permanent tooth.

- Use PM-04 Pediatric Pain Control for patients age 2 through younger than 16 years. Contact Medical Control for analgesic direction for a patient younger than 2 years. Maintain close airway observation when facial swelling, bleeding, or loose teeth are present.

Related pathways / contacts

UP-11 Adult Pain Control - Use for patients age 16 years or older.

PM-04 Pediatric Pain Control - Use for patients age 2 through younger than 16 years.

Source and references

Tennessee EMS BLS/ALS State Protocol Guidelines 2024-2025 - SOP 403 Avulsed Teeth | American Academy of Pediatric Dentistry, Acute Management of an Avulsed Permanent Tooth, Reference Manual 2025-2026 | International Association of Dental Traumatology Guidelines for Avulsion of Permanent Teeth, endorsed by AAPD | Claiborne Covenant EMS Formulary - current medical-director source workbook | Claiborne County EMS UP-07 source PDF - Dental Problems, revised September 1, 2025 | Claiborne County EMS UP-11 Adult Pain Control, PM-04 Pediatric Pain Control, and applicable adult cardiac and airway protocols

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